



Department of Defense
Defense Health Agency
Performance Work Statement

Newborn Screening Services

Naval Hospital Jacksonville, FL 32214
Naval Hospital Guantanamo, Bay Cuba, FPO, AA 34009

1.0 GENERAL INFORMATION

1.1 This is a non-personal services contract to provide comprehensive, timely, and accurate Newborn Screening Services (NBS) in support of Naval Hospital Jacksonville and Naval Hospital Guantanamo Bay, Cuba. Services shall include collection kit provision, laboratory testing, result reporting, and consultation on follow-up care for abnormal results in accordance with federal and state guidelines.

1.2 **Description of services/introduction:** The contractor shall provide all personnel, equipment, supplies, facilities, transportation, tools, materials, supervision, and other items and non-personal services necessary to perform Newborn Screening Services as defined in this Performance Work Statement (PWS). All services shall be performed in a manner consistent with high standards of laboratory practice and support the readiness and health of the military family population served by Naval Hospital Jacksonville, Florida and Naval Hospital Guantanamo Bay, Cuba.

1.3 **Background:** On 01 May 2021, Naval Hospital Jacksonville awarded contract N6264521D5002 to the Florida Department of Health for Newborn Screening Services. The period of performance of the contract is 01 May 2021 through 30 April 2026. Naval Hospital Jacksonville, FL and Guantanamo Bay serve a large and diverse beneficiary population, including active-duty service members, retirees, and their dependents. As part of the Department of Defense healthcare system, Military Treatment Facilities with Labor and Delivery (L&D) units are required to comply with military, federal, and state public health regulations regarding newborn screening. Ensuring early detection of congenital and metabolic disorders is critical for readiness and the long-term health of military families.

1.4 **Scope:** The contractor shall provide comprehensive newborn screening services for Naval Hospital Jacksonville, FL, and Naval Hospital Guantanamo Bay, Cuba. The Government reserves the right to add additional Military Treatment Facilities where Florida State services may be required, as necessary, via bilateral modification. Services include the supply of blood specimen collection kits, laboratory analysis of dried blood spot samples, and timely electronic reporting of results. Services shall cover all conditions required by the Department of Health and Human Services Recommended Uniform Screening Panel (RUSP), such as metabolic, endocrine, and genetic disorders. The contractor shall ensure daily specimen receiving, immediate notifications of abnormal results, and clinical consultation with qualified specialists to support follow-up care. Training and technical assistance for hospital staff on specimen collection and compliance requirements shall also be provided. All services shall be performed off-site in a CLIA-certified, state-licensed laboratory and comply with HIPAA, DoD, DHA data security standards, and be capable of processing specimens in accordance with Clinical and Laboratory Standards Institute (CLSI) guidelines and Department of Health and Human Services' Recommended Uniform Screening Panel (RUSP).

1.5 **Period of Performance (PoP):** The contractor shall begin services per the Performance Work Statement on 01 May 2026. The Period of Performance has a Base Period of 01 May 2026 through 30 April 2027, including four optional periods/years.

Option Period 1: 01 May 2027 through 30 April 2028
Option Period 2: 01 May 2028 through 30 April 2029
Option Period 3: 01 May 2029 through 30 April 2030
Option Period 4: 01 May 2030 through 30 April 2031
Option Period 5 (-8): 01 May 2031 through 31 October 2031

1.6 Administrative specifications

1.6.1 Place of performance: The contractor shall perform newborn screening testing services through its designated reference laboratory.

Specimens collected by the Military Treatment Facilities - Naval Hospital Jacksonville and Naval Hospital Guantanamo Bay, Cuba shall be received for initial processing and forwarded to the contractor's designated reference laboratory for newborn screening testing. The Government reserves the right to add additional Military Treatment Facilities where Florida State services may be required, as necessary, via bilateral modification. All testing and result reporting shall be performed at the reference laboratory located at:

Florida Department of Health
Division Of Disease Control & Health Protection
Bureau of Public Health Laboratories – Jacksonville
1217 N Pearl St
P.O Box 210
Jacksonville, FL 32202

1.6.2 **Recognized Federal holidays:** The contractor is not required to conduct business on Federally recognized holidays - Thanksgiving Day, Christmas Day, and New Year's Day and Presidential Executive Orders.

1.6.3 **Hours of operation:** The contractor shall maintain normal business operations Monday through Friday, from 0800-1700. A qualified nurse shall be available on Saturdays to support time-critical call outs. The contractor shall be closed on Sundays and on state-recognized holidays or when the Government facility is closed due to local or national emergencies, administrative closings, or similar Government directed facility closings. The contractor shall, at all times, maintain an adequate workforce to ensure the uninterrupted performance of all tasks defined within this PWS, except during periods when the Government facility is closed for the reasons stated above.

1.7 Quality

1.7.1 Quality Control (QC): The contractor shall develop and maintain an effective QC program designed for medical surveillance to continuously monitor and improve quality of specimen collection, submission of required demographic information from submitting facilities, specimen analysis, and reporting of results in accordance with this PWS. Quarterly review or report to be provided. The contractor shall develop and implement procedures to identify, prevent, and ensure nonrecurrence of defective services. The contractor's QC program is the means by which the work complies with stated requirements. The QCP is to be delivered with the contractor's proposal and comprehensive written QCP shall be submitted to the CO and COR within 5 working days when changes are made thereafter. After acceptance of the Quality Control Plan (QCP) the contractor shall receive the CO's acceptance in writing of any proposed change to his QC system. See Part 7, Technical Exhibit 1 - CDRL A001.

1.7.2 Quality assurance (QA): The government will evaluate the contractor's performance under this contract in accordance with the Quality Assurance Surveillance Plan (QASP). This plan provides a systematic method for the Government to evaluate performance and to ensure that the contractor has performed in accordance with the performance standards. It defines how the performance standards will be applied, the frequency of surveillance, and the minimum acceptable defect rate(s).

1.7.3 The contractor shall participate in and maintain compliance with the Centers for Disease Control and Prevention's (CDC) quarterly external proficiency testing program for Newborn Screening (dried blot methodology) and external proficiency testing programs for all Centers for Medicare and Medicaid Services (CMS) regulated testing procedures/analytes.

1.8 Contractor Personnel

1.8.1 Contractor employees shall not be supervised, directed, or evaluated by government personnel. The contractor shall solely be responsible for the hiring, supervision, performance and compensation of its personnel.

1.9 Reporting

1.9.1 Non-Disclosure Agreement (NDA): All contractor personnel who will obtain access to proprietary, classified, or confidential information or any information release of which is protected or governed by law or regulation associated with DHA acquisitions shall be required to complete and sign a DHA contractor NDA (DHA Form 49) prior to beginning work on the subject contract. The contractor shall execute an NDA on behalf of the company and shall ensure that all staff assigned to, including all subcontractors and consultants, or other personnel performing on contract/Task order execute an NDA protecting the procurement sensitive information of the Government and the proprietary information of other contractors. The NDA shall be executed not later than first day of employment and to be renewed upon exercising a contract option period. Assignment of staff who have not executed this statement or failure to adhere to this statement shall constitute default on the part of the contractor. The contractor shall maintain originally signed NDAs of individual employees and provide copy to the COR.

1.9.2 Government's COR: The COR monitors all technical aspects of the contract and assists in contract administration. The COR is authorized to perform the following functions: assure that the contractor performs the technical requirements of the contract; perform inspections necessary in connection with contract performance; maintain written and oral communications with the contractor concerning technical aspects of the contract; issue written interpretations of technical requirements, including Government drawings, designs, specifications; monitor contractor's performance and notifies both the CO and contractor of any deficiencies; coordinate availability of government furnished property; and provide site entry of contractor personnel. A letter of designation issued to the COR, a copy of which is sent to the contractor, states the responsibilities and limitations of the COR, especially with regard to changes in cost or price, estimates or changes in delivery dates. The COR is not authorized to change any of the terms and conditions of the resulting contract.

1.9.3 Post award conference/periodic progress meetings: The contractor agrees to attend any post award conference convened by the contracting activity or contract administration office in accordance with FAR Subpart 42.5. The CO, COR, and other Government personnel, as appropriate, may meet periodically with the contractor to review the contractor's performance. At these meetings the CO will appraise the contractor of how the government views the contractor's performance and the contractor will apprise the Government of problems, if any, being experienced. Appropriate action shall be taken to resolve outstanding issues. These meetings shall be at no additional cost to the government.

1.10 Contractor Identification

1.10.1 Contractor personnel performing services in a contractor capacity in a Government facility are required to possess and wear an identification badge that displays his or her name and the name of their company. All contractor personnel shall identify themselves as contractor support personnel in all forms of communication with all entities with whom DHA/Deputy Assistant Director for Acquisition (DAD-A)/Head of the Contracting Activity (HCA) has business dealings. The contractor shall: Answer all telephone calls and have a personalized voice message with an introductory statement that includes the fact that the person is contractor support personnel. Ensure all those with whom the person interacts in any face-to-face dealings while supporting the DAD-A understands that the person is contractor support

personnel. Include a title block in all emails that states the fact that the person is contractor support personnel. Ensure all those with whom the person interacts in any face-to-face dealings while supporting DHA/DAD-A/HCA understands that the person is contractor support personnel.

1.10.2 Contractor personnel will be required to attend meetings or otherwise communicate with Government and/or other contract representatives to meet the requirements of this order. Contractor personnel shall make their contractor status known during introductions.

1.10.3 Contractor personnel, while performing in a contractor capacity, are prohibited from using their retired or reserve component military rank or title in any written or verbal communications associated with the contracts in which they provide services.

PART 2

2.0 DEFINITIONS, ACRONYMS, AND APPLICABLE PUBLICATIONS/INSTRUCTIONS

2.1 Definitions:

2.1.1 Category D: Information Technology (IT) and Telecommunications Services (called D-Services)

2.1.2 Category R: Support (Professional/Administrative/Management) Services (called R-Services)

2.1.3 Contracting Officer (CO): A person with the authority to enter into, administer, and/or terminate contracts and make related determinations and findings.

2.1.4 Contracting Officer's Representative (COR): An individual, including a contracting officer's technical representative (COTR), designated and authorized in writing by the CO to perform specific technical or administrative functions. This individual does NOT have authority to change the terms and conditions of the contract.

2.1.5 Nonpersonal services contract: a contract under which the personnel rendering the services are not subject, either by the contract's terms or by the manner of its administration, to the supervision and control usually prevailing in relationships between the Government and its employees.

2.1.6 Quality Assurance Surveillance Plan (QASP): An organized written document specifying the surveillance methodology to be used for surveillance of contractor performance. The Government may either prepare the QASP or require the offerors to submit a proposed quality assurance surveillance plan for the Government's consideration in development of the Government's plan.

2.2 Acronyms: *(List all acronyms used in the PWS and what they represent. At a minimum, insert the acronyms provided below).*

AIS	Automated Information System
APL	Approved Products List
AQL	Acceptable Quality Level
ARRT	Acquisition Requirements Roadmap Tool
ATO	Authority to Operate
B2B	Business-2-Business
CAC	Common Access Card
CAP	Cloud Access Point
CCEVS	Common Criteria Cybersecurity Evaluation and Validation Scheme

CDI	Covered Defense Information
CE	Computer Environment
CDRL	Contract Data Requirement List
CIO	Chief Information Officer
CJCSM	Chairman of the Joint Chiefs of Staff Manual
CMMC	Cybersecurity Maturity Model Certification
CMR	Contractor Manpower Reporting
CNSSI	Committee on National Security Systems Instruction
CO	Contracting Officer(s)
CONUS	Continental United States (excludes Alaska and Hawaii)
COR	Contracting Officer Representative
COTR	Contracting Officer's Technical Representative
CSP	Cloud Service Provider
CSSP	Cyber Security Service Provider
CUI	Controlled Unclassified Information
DAD-A	Deputy Assistant Director for Acquisition
DC3	DoD Cyber Crime Center
DD Form 254	Department of Defense Contract Security Requirement List (if applicable)
DB	Design-Build
DBB	Design-Bid-Build
DFARS	Defense Federal Acquisition Regulation Supplement
DHA	Defense Health Agency
DISA	Defense Information System Agency
DoD	Department of Defense
DoDD	Department of Defense Directive
DoDI	Department of Defense Instruction
DSAs	Data Sharing Agreements
DSAA	Data Sharing Agreement Application
DMZ	Demilitarized Zone
DoDM	Department of Defense Manual
DPCLO	DHA Privacy and Civil Liberties Office
DUA	Data Use Agreement
eMSM	Enhanced Multi-Service Markets
EULA	End User License Agreement
EVM	Earned Value Management
FAR	Federal Acquisition Regulation
FCI	Federal contract information
FE	Facilities Enterprise
FedRAMP	Federal Risk Authorization and Management Program
FISMA	Federal Information Security Modernization Act
FRCS	Facility Related Control Systems
FSO	Facilities Security Officer
HA	Health Affairs
HIPAA	Health Insurance Portability and Accountability Act
HCA	Head of the Contracting Activity
HIT	Health Information Technology
IGCE	Independent Government Cost Estimate
IA	Information Assurance
IO	Initial Outfitting
I/O	In/Out Processing Portal
IPV	Internet Protocol Version

IS	Information System
ISP	Internet Service Provider
IT	Information Technology
ISCM	Information Security Continuous Monitoring
IV&V	Independent Verification & Validation
MedCOI	Medical Community of Interest
MHS	Military Health System
MIL-STD	Military Standard
MTFs	Military Treatment Facilities
NCR	National Capitol Region
NDA	Non-Disclosure Agreement
NIAP	National Information Assurance Partnership
NIST	National Institute of Standards and Technology
OCONUS	Outside Continental United States (includes Alaska and Hawaii)
ODC	Other Direct Costs
OPM	Office of Personal Management
OSD	Office of the Secretary of Defense
P-ATO	Personal Authorization to Operate
P&R	Personnel and Readiness
PGI	Procedures, Guidance and Information
PDT	Project Delivery Team
PHI	Protected Health Information
PII	Personally Identifiable Information
PIT	Platform Information Technology
PK	Public Key
PKI	Public Key Infrastructure
POA&M	Plan of Action and Milestones
POC	Point of Contact
PMO	Program Management Office
PoP	Period of Performance
PP	Personal Property
PPSM	Ports, Protocols, and Services Management
PRS	Performance Requirements Summary
PSP	Personnel Security Program
PWS	Performance Work Statement
QA	Quality Assurance
QAP	Quality Assurance Program
QASP	Quality Assurance Surveillance Plan
QC	Quality Control
QCP	Quality Control Plan
RFP	Request for Proposal
RFQ	Request for Quotation
RMF	Risk Management Framework
SP	Special Publication
SPRS	Supplier Performance Risk System
SRM	Sustainment, Restoration and Modernization
SRG	Security Requirements Guides
STIG	Security Technical Implementation Guides
TOS	Terms of Service
US	United States
UFC	Unified Facilities Criteria

VPN
XML

Virtual Private Network
Extensible Markup Language

PART 3

3.0 GOVERNMENT FURNISHED PROPERTY, EQUIPMENT, AND SERVICES

The Requiring Activity Authority has assessed the need for Government Furnished Property, Equipment, and Services and determined:

3.1 Services: The Government:

☒ Will **NOT** provide Government Furnished Services in support of this contract/task order. As a result, this paragraph is Not Applicable.

☐ **WILL** provide Government Furnished Services required in support of this contract/task orders. These Services are described below: Not applicable.

3.2 Facilities: The Government:

☒ Will **NOT** provide Facilities in support of this contract/task order. As a result, this paragraph is Not Applicable.

☐ **WILL** provide Facilities in support of this contract/task orders. The Government provided Facilities are described below: Not applicable.

3.3 Utilities: The Government:

☒ Will **NOT** provide Utilities in support of this contract/task order. As a result, this paragraph is Not Applicable.

☐ **WILL** provide Utilities in support of this contract/task orders. The Government provided Utilities are described below: Not applicable.

3.4 Equipment: The Government:

☒ Will **NOT** provide Equipment in support of this contract/task order. As a result, this paragraph is Not Applicable.

☐ **WILL** provide Equipment in support of this contract/task orders. The Government provided Equipment is described below: Not applicable.

3.4.1 Procurement Integrated Enterprise (PIEE), GFP Module Application. Not applicable.

3.5 Materials: The Government:

☒ Will **NOT** provide Materials in support of this contract/task order. As a result, this paragraph is Not Applicable.

☐ **IS** providing Materials in support of this contract/task orders. The Government-provided Materials are described below: Not applicable.

PART 4

4.0 CONTRACTOR FURNISHED ITEMS AND SERVICES

4.1 Services: The Contractor:

☒ Will **NOT** provide Contractor Furnished Services in support of this contract/task order. As a result, this paragraph is Not Applicable.

☐ **WILL** provide Contractor Furnished Services required in support of this contract/task orders. These Services are described below: Not Applicable.

4.2 General: The contractor shall furnish all supplies, equipment, facilities and services required to perform work listed under Section 5 of this PWS.

4.3 Materials: The contractor shall provide a specimen collection kit and a special filter paper card in accordance with the contractor's standard practice.

4.4 Equipment: Not applicable.

4.5 Facilities: The contractor shall store residual dried blood spots at a facility that provides security, confidentiality, stability of temperature and humidity, and retrievability for six (6) months.

PART 5

5.0 SPECIFIC TASKS

5.1 Specimen Collection Material Supply and Specimen Transport

5.1.1 The contractor shall provide a specimen collection kit and a special filter paper card in accordance with the contractor's standard practice.

5.1.2 The MTF will provide the contractor with the filter paper card collected from all newborns by heel-stick between 24-48 hours of age, but not greater than the 4th day of life unless the clinical situation makes later collection necessary. If collected prior to 24 hours, the facility shall submit a second specimen in accordance with the contractor's standard practice.

5.1.3 The MTF will complete the test request form (TRF) for Newborn screening, which contains fields for recording critical information about the baby.

5.1.4 Transport of specimens collected and to be tested by the contractor will be shipped by the MTF in accordance with the contractor's standard practice.

5.1.5 Specimen shall be picked up daily by courier and transported to the contractor's facility for analysis. Normal screening results shall be reported electronically with 3 to 5 business days of specimen receipt,

while abnormal or critical results shall be reported immediately to the designated hospital staff via secured electronic communication and follow-up phone notification.

5.1.6 The contractor shall retain dried blood spots for at least six months from date of test.

5.2 Training. Prior to submission of samples and as requested by the MTF during the life of this contract, the contractor shall provide ongoing training, education and technical support for Naval Hospital Jacksonville, FL and Guantanamo Bay, Cuba staff to ensure proper specimen collection, documentation, and compliance with state and federal reporting requirements. Training shall include sample shipment preparation and packaging, logistical coordination for sample processing, and physician education. Data management shall comply with HIPAA, DoD Cybersecurity and DHA Health IT standards. Such training, education, or technical assistance may be provided in person, by phone, e-mail, webcam, video teleconference, videotape, or letter, as appropriate, to improve quality of specimen submission, decrease sample collection and patient identification errors, and reduce the need for newborn re-screens.

5.2.1 The contractor shall not be compensated separately for training.

5.3 Screening Tests

5.3.1 The contractor shall provide the MTF Newborn screening services to include laboratory testing for the detection of a core set of conditions and report results for additional disorders detected by this testing. The conditions to be tested shall include those on the Recommended Uniform Screening Panel (which is hereby incorporated as reference and can also be found at <https://www.hrsa.gov/advisory-committees/heritable-disorders/rusp>), as recommended by the Secretary's Advisory Committee on Heritable Disorders in Newborns and Children with the approval of United States Department of Health and Human Services and as approved by state law. Any conditions that are added to the Recommended Uniform Screening Panel throughout this contract period shall be added to the set of conditions screened under this contract as they are able to be incorporated and validated by the contractor. The contract may be modified if the cost of screening increases to include those conditions and or/if testing technologies change and/or improve.

5.3.2 The MTF shall be given a minimum of 180-days advance notice of a change of testing (Addition/Deletion/Modification of screened disorders and/or screening technologies). Contractor notification will consist of (at the minimum) an official letter, sent on contractor official departmental letterhead to the Contracting Officer, addressed to the MTF laboratory medical director and laboratory administrator. Additional disorders may be added as they are incorporated into the state's Newborn Screening program and/or new disorders become prevalent. Changes in testing methodology may occur without notice, such as the addition or elimination of ratios, changes in analyte cut-offs, etc.

5.4 Laboratory Testing and Analysis

5.4.1 The contractor shall provide newborn screening for all the disorders as indicated in paragraph 5.4.2. In addition, when appropriate for initial screening or secondary testing, biochemical and molecular/genetic tests shall be performed to clarify screening results, according to the contractor's established protocols.

5.4.2 The contractor shall be responsible for ensuring that all required test panels are performed, including but not limited to tests for inborn errors of metabolism, hemoglobinopathies, endocrine disorders, cystic fibrosis, and severe combined immunodeficiency (SCID).

5.4.3 Repeat testing and/or confirmatory tests on the initially collected patient specimen shall be performed as appropriate if a test does not fall within normal limits on the initial screening test.

5.4.4 The contractor shall provide re-screening of newborns in cases of improper sample collection, technical problems such as patient identification errors or degradation of specimens during shipment/transport.

5.4.5 The contractor shall provide re-screening of all infants weighing less than 1500 grams at birth, and premature and sick infants requiring prolonged hospitalization according to the contractor's established protocols.

5.5 Case Management for Screens

5.5.1 The contractor shall perform the required tests and then provide the results of the screen by electronic post to authorized personnel at the MTF. Results shall be reported within 3-5 days of sample receipt. Results will be transmitted through MoveIt, the contractor's secure, encrypted file-transfer system used for the official delivery of results. In addition, results will also be available through the contractor's website for authorized users in accordance with the contractor's standard practice and as agreed upon by the MTF.

5.5.2 Timelines and methods of delivery of presumptive positive/significant abnormal results will follow established protocols per disorder, age of child and degree of illness.

5.5.3 Abnormal results of an urgent nature will be reported by phone, as well as by secure email, electronic post, and through encrypted file transfer (MoveIt) to the MTF newborn screening coordinator and the health care provider (as indicated on the screening card or by medical records at the MTF). Appropriate recommendations for further testing or follow-up will be made.

5.5.4 The contractor shall monitor the newborn's case until screening results are reported, or diagnosis is confirmed.

5.6 Specialty Care for Positive Screens

5.6.1 A specialist in metabolic genetics, immunology and hematology will be available for consultation by phone or email to advise patient management, if required. Contact information for a specialist in pulmonology or endocrinology will be made available if indicated.

5.6.2 The contractor shall provide access to board-certified geneticists or equivalent clinical specialists to consult with Naval Hospital Jacksonville on abnormal results, including recommendations for follow-up testing, referrals, and care coordination.

5.6.3 Specialists and/or the contractor's newborn screening coordinators will assist in establishing appropriate follow up of screen positive babies.

5.6.4 The contractor shall respond within the next business day from receipt of any particular question(s) by the MTF Healthcare Providers that may not have been covered in the Laboratory testing and provide quality customer service herein to the terms of the intent and terms of this contract.

5.6.5 The contractor shall contact the respective MTF laboratory to ensure a positive transfer of patient results (for all results, both normal and abnormal).

5.6.5.1 The MTF will record the Newborn Screen results into the appropriate Medical Records database (e.g., Composite Health Care System (CHCS), Health Artifact and Image Management Solution (HAIMS), etc.)

5.6.5.2 As part of continued follow-on care, the communication between the contractor and the MTF shall continue regularly without prompting from the MTF.

5.6.5 The contractor shall not be compensated separately for Specialty Care for Positive Screens.

5.7 Other Responsibilities

5.7.1 The contractor shall provide monthly summary reports and an annual performance review of screening outcomes, turnaround times, and quality assurance metrics.

5.7.2 The contractor shall keep the Contracting Officer Representative (COR) informed of the most recent developments in clinical laboratory methods and medical interpretation through periodic newsletters, reports and data sheets.

5.7.3 The contractor shall have reference laboratory representatives available to assist in problem solving at no additional cost to the Government with access to knowledgeable laboratory professionals with minimal call transfers and holding.

5.7.4 The contractor shall provide monthly summary reports and an annual performance review of screening outcomes, turnaround times, and quality assurance metrics. All services shall be performed in a manner consistent with high standards of laboratory practice and support the readiness and health of the military family population served by Naval Hospital Jacksonville, Florida and Naval Hospital Guantanamo Bay, Cuba.

5.7.5 Residual dried blood spots shall be stored at a facility that provides security, confidentiality, stability of temperature and humidity, and retrievability for six months. Stored specimens shall be accessible to the Department of Defense (DoD) beneficiaries throughout the storage period. The contractor shall retrieve and provide stored specimens to the DoD within 7 days upon request. If it is the contractor's practice to dispose of filter paper blood spots following the six-month period, the left-over filter paper blood spots shall be disposed of, and it shall be in a manner that ensures confidentiality. No filter paper blood spots can be used for research without the explicit consent of the newborn's parent/legal guardian, and approval by a DoD approved Institutional Review Board. Documentation of the storage, use and disposal of residual dried blood spots shall be maintained and shall be made available to the DoD upon request.

5.8 **Volume.** Quantity amounts for CLINs are estimated based on historical data and are subject to fluctuation. The Government reserves the right to reevaluate quantities, if necessary.

PART 6

6.0 INFORMATION TECHNOLOGY & SECURITY

6.1 All work under this contract is unclassified.

6.2 The TIER 1 or TIER 2 levels and position sensitivity designation for positions under this contract is: Reserved.

6.3 Personally Identifiable Information (PII)/Protected Health Information (PHI), Procurement, and Federal information requirements: Reserved.

6.4 Training: Reserved.

6.5 Cybersecurity Requirements for Non-DoD IT or Covered Contractor IS: Reserved.

6.6 Risk Management Framework (RMF) for DoD IT: Reserved.

6.7. Facility Related Control Systems: Reserved.

6.8. System Communications: Reserved.

PART 7, ATTACHMENT 1 PERFORMANCE REQUIREMENTS SUMMARY

The contractor service requirements are summarized into performance objectives that relate directly to mission essential items. The performance threshold briefly describes the minimum acceptable levels of service required for each requirement. These thresholds are critical to mission success.

Performance Objective (The Service required—usually a shall statement from Part 5)	PWS Reference	Acceptable Quality Level (AQL) (This is the maximum error rate. It could possibly be “Zero deviation from standard”)	Method of Surveillance
Performance Requirements Summary (PRS) # 1. Specimen Collection and Transport	5.1	100% of the Performance Period	100% inspection at MTF
PRS # 2 Training	5.2	100% of the Performance Period	100% inspection at MTF
PRS # 3 Screening Test Changes	5.3.2	100% of the Performance Period	100% inspection at MTF
PRS # 4 Lab Analysis	5.4	100% of the Performance Period	100% inspection at MTF
PRS # 5 Test Results Timeline	5.1.5	100% of the Performance Period	100% inspection at MTF
PRS # 6 Test Results Method of delivery	5.1.5, 5.5.1	100% of the Performance Period	100% inspection at MTF

PART 7, TECHNICAL EXHIBIT 1
DELIVERABLES SCHEDULE

<u>Deliverable</u>	<u>Frequency</u>	<u>Medium/Format</u>	<u>Submit To</u>	<u>Applicable to:</u>
CDRL A001 – Quality Control Plan  CDRL 1 Quality Plan	Within 30 days of task order award	Contractors' discretion	CDRL A001 to CO	All, See PWS 1.7.1
Summary Reports	Monthly	Contractors' discretion	COR	PWS 5.7.1
Performance Review	Annually	Contractors' discretion	COR	PWS 5.7.1